

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Randomized Open Label Phase II Study of Immune Checkpoint Inhibitor Combinations With Axitinib in Patients With Previously Untreated Locally Advanced Unresectable or Metastatic Renal Cell Carcinoma
Short Title:	A Study of Immune Checkpoint Inhibitor Combinations With Axitinib in Participants With Untreated Locally Advanced Unresectable or Metastatic Renal Cell Carcinoma
Protocol Number:	661000798459730620
NCT Number:	NCT05805501
Sponsor Name:	Roche
Investigational Product:	axitinib
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Incidence and Severity of Adverse Events (AEs)
Secondary Obj.:	Not Provided

2.2 Background & Rationale

This study will evaluate the safety of tobemstomig (RO7247669) in combination with axitinib alone or with tiragolumab (anti-TIGIT) and axitinib as compared to pembrolizumab and axitinib in participants with previously untreated, unresectable locally advanced or metastatic clear-cell renal cell carcinoma (ccRCC).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: ACTIVE_NOT_RECRUITING
Target N:	199

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Eastern Cooperative Oncology Group Performance Status (ECOG PS) of 0 or 1
- * International Metastatic RCC Database Consortium (IMDC) risk intermediate (score of 1 or 2) or poor (score of 3-6)
- * Measurable disease with at least one measurable lesion
- * Histologically confirmed ccRCC with or without sarcomatoid features
- * Negative for HIV, hepatitis B, or hepatitis C virus (HCV)

4.2 Exclusion Criteria

- * Pregnant or breastfeeding, or intention of becoming pregnant during the study or within 90 days after the final dose of tiragolumab, 4 months after the final dose of tobemstomig (RO7249669) and pembrolizumab, or for 1 week after the

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final dose of axitinib, whichever occurs last

- * Inability to swallow a tablet or malabsorption syndrome
- * Prior treatment for localized and/or metastatic RCC with systemic RCC-directed therapy, including T-cell costimulating or immune checkpoint blockade therapies
- * Ongoing use or anticipated need for treatment with a strong CYP3A4/5 inhibitor or inducer
- * Major surgical procedure, other than for diagnosis, within 4 weeks prior to initiation of study treatment, or anticipation of need for a major surgical procedure during the study
- * Uncontrolled or symptomatic hypercalcemia or symptomatic hypercalcemia requiring continued use of bisphosphonate therapy or denosumab
- * Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases
- * History of leptomeningeal disease
- * Uncontrolled tumor-related pain
- * Uncontrolled pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures (once monthly or more frequently)
- * Moderate to severe hepatic impairment (Child-Pugh B or C)
- * Uncontrolled hypertension
- * Prior history of hypertensive crisis or hypertensive encephalopathy
- * Significant cardiovascular/cerebrovascular disease within 3 months (12 months for UK participants) prior to randomization
- * History of clinically significant ventricular dysrhythmias or risk factors for ventricular dysrhythmias
- * History of congenital QT syndrome
- * Resting heart rate (HR) ≥ 100 bpm (or clinically significant tachycardia)
- * Stroke (including transient ischemic attack), myocardial infarction, or other symptomatic ischemic event, or thromboembolic event (e.g., deep venous thrombosis [DVT], pulmonary embolism [PE]) within 3 months (12 months for UK participants) before randomization
- * Significant vascular disease (e.g., aortic aneurysm or arterial dissection requiring surgical repair or recent peripheral arterial thrombosis) within 6 months prior to Day 1 of Cycle 1
- * Tumors invading pulmonary blood vessels, cavitating pulmonary lesions or known endobronchial disease
- * Tumor invading the gastrointestinal (GI) tract, including abdominal or tracheoesophageal fistulas
- * Evidence of abdominal free air not explained by paracentesis or recent surgical procedure
- * Active peptic ulcer disease, acute pancreatitis, acute obstruction of the pancreatic or biliary duct, appendicitis, cholangitis, cholecystitis, diverticulitis, gastric outlet obstruction
- * Intra-abdominal abscess within 6 months before initiation of study treatment
- * Clinical signs or symptoms of GI obstruction or requirement for routine parenteral hydration, parenteral nutrition, or tube feeding
- * Evidence of bleeding diathesis or significant coagulopathy
- * Grade ≥ 3 hemorrhage or bleeding event within 28 days prior to initiation of study treatment
- * Clinically significant hematuria, hematemesis, hemoptysis of ≥ 0.5 teaspoon (2.5 mL) of red blood, coagulopathy, or other history of significant bleeding (e.g., pulmonary hemorrhage) within 3 months before initiation of study treatment
- * Active or history of autoimmune disease or immune deficiency

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- * Treatment with systemic immunosuppressive medication within 2 weeks prior to initiation of study treatment, or anticipation of need for systemic immunosuppressive medication during study treatment
- * Prior allogeneic stem cell or solid organ transplantation
- * History of idiopathic pulmonary fibrosis, organizing pneumonia (e.g., bronchiolitis obliterans), drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis on screening chest computed tomography (CT) scan
- * History of another primary malignancy other than RCC within 2 years prior to screening, with the exception of malignancies with a negligible risk of metastasis or death (e.g., 5-year OS rate $\geq 90\%$)
- * Administration of a live, attenuated vaccine within 4 weeks before randomization or anticipation that such a live, attenuated vaccine will be required during the study
- * Active tuberculosis (TB)
- * Severe infection within 4 weeks prior to initiation of study treatment
- * Participants with active Epstein-Barr virus (EBV) infection or known or suspected chronic active EBV infection at screening
- * Treatment with therapeutic oral or IV antibiotics within 2 weeks prior to initiation of study treatment
- * Known hypersensitivity to Chinese hamster ovary cell products or to any component of tremelimumab, tiragolumab, pembrolizumab, or axitinib

11. Study Identifiers & Governance

Primary ID: 661000798459730620

Registry Number: NCT05805501

Sponsor: Roche

Study Title: A Study of Immune Checkpoint Inhibitor Combinations With Axitinib in Participants With Untreated Locally Advanced Unresectable or Metastatic Renal Cell Carcinoma

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____